Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
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INFORMED CONSENT FORM

Submandibular Gland Excision

Removal of the salivary gland under the jaw

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Side: ☐ Right ☐ Left

Reason for surgery: ☐ Salivary stone / recurrent infection ☐ Lump in the gland ☐ Other _____

Nerve monitoring: ☐ Will be used ☐ Will not be used

Imaging / biopsy result: _____

1 Your condition and the aim of the procedure

The submandibular gland is a salivary gland located just below the lower jaw on each side, which releases saliva into the mouth. It produces a significant share of the saliva; however, because the gland on the other side and the salivary glands in front of the ears and under the tongue continue to work, **removing one gland does not cause a dry mouth in most patients**.

This operation is most often performed because of **salivary gland stones** and the **recurrent swelling and infections** they cause. A stone blocks the flow of saliva into the mouth, leading to painful swelling under the jaw when eating and repeated infections. Less often, a **lump (tumour)** in the gland requires surgery. The proportion of malignant tumours is higher in submandibular gland lumps than in parotid lumps; removing the gland and examining it under the microscope is therefore important for a definitive diagnosis.

Three nerves run close to the gland: the nerve branch that moves the lower lip, the nerve that gives sensation to the tongue and the nerve that moves the tongue. The aim of the operation is to remove the gland completely while preserving these nerves.

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2 How is the procedure performed?

The operation is performed under general anaesthesia in an operating theatre.

1. **Incision:** An incision, usually 4–6 cm long, is made about 2–3 cm below the jawbone, parallel to a natural skin crease of the neck. Placing the incision well below the jaw helps to protect the nerve branch that moves the lower lip.
2. **Protecting the nerves:** The thin muscle layer under the skin (platysma) is opened. The nerve branch that moves the lower lip (marginal mandibular nerve) is protected, either by seeing it or by staying in a safe tissue layer.
3. **Removing the gland:** The gland is separated from the surrounding tissues and the vessels supplying it are tied. The salivary duct is carefully separated from the nerve that gives sensation to the tongue (lingual nerve), then tied and divided. The nerve that moves the tongue (hypoglossal nerve) is preserved. If there is a stone in the duct, as much of the far part of the duct as possible is also removed.
4. **Pathological examination:** The removed gland is sent to pathology; the result is usually available within 1–2 weeks.
5. **Closure:** Bleeding is controlled, a thin **drain** is usually placed and the skin is closed with cosmetic stitches.

If there is a stone inside the mouth close to where the duct opens, it may sometimes also be removed separately from inside the mouth. This will be discussed with you before surgery.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia	About 1–2 hours	Usually 1 night; discharge once the drain is removed

3 Expected benefits and likelihood of success

- An end to recurrent painful swelling and infections caused by stones
- No more need for repeated courses of antibiotics
- Removal of a lump in the gland and a definitive diagnosis

After surgery for stones and chronic infection, symptoms resolve completely in **more than 90%** of patients. If a stone remains in, or re-forms in, the part of the duct left in the mouth, swelling of the floor of the mouth may rarely continue. For benign lumps, surgery is usually a permanent cure; malignant tumours may need further treatment.

4 Other treatment options

- **Medical and supportive treatment:** Plenty of fluids, sour tastes to stimulate saliva, massage, warm compresses and antibiotics during infections. This may ease symptoms but does not remove the stone.
- **Removing the stone through the mouth:** Stones in the part of the duct near the mouth that can be felt may be removed through a small incision inside the mouth, preserving the gland.
- **Salivary duct endoscopy (sialendoscopy):** Stones of suitable size and position can be removed from inside the duct with a fine endoscope; this is not available in every centre or suitable for every stone.
- **Observation:** For small lumps thought to be benign, observation may rarely be discussed; however, surgery is usually recommended for a definitive diagnosis in this gland.

5 What happens if the procedure is not performed?

Without surgery, swelling and infections due to stones may continue to recur; rarely an abscess (collection of pus) may develop around the gland, requiring emergency treatment. If there is a lump in the gland, a definitive diagnosis cannot be made; an untreated malignant tumour may progress.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

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6 Preparing for the procedure

- Before surgery, blood tests and imaging (ultrasound and, if necessary, CT/MRI) are carried out; you will be assessed by the anaesthetist.
- You must not eat or drink anything for **6–8 hours** before the operation (please follow the anaesthetist's instructions).
- If the gland is actively infected, surgery is usually planned once the infection has been treated.
- For male patients, the beard in the incision area may be trimmed on the day of surgery.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 24–48 hours	There will be mild to moderate pain, tightness and swelling under the jaw, controlled with painkillers. Swallowing may be slightly uncomfortable. The drain is usually removed on day 1. Sleeping with your head raised is recommended. You can start eating soft foods straight away.
Week 1	You can return to daily activities. Stitches are usually removed on **days 5–7**. Numbness above the incision, along the jawline and around the incision is normal. The pathology result will be shared with you during this period or shortly afterwards.
Weeks 2–4	Most people return to desk work within 1 week. Strenuous exercise and heavy lifting should be postponed for about **2 weeks**. Firmness and slight swelling at the incision gradually decrease.
Months 2–6	The scar gradually fades and often becomes inconspicuous in the neck crease. Any temporary nerve weakness or reduced sensation usually recovers during this period.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Pain, tightness and swelling under the jaw; slight discomfort when swallowing
- Numbness of the skin around and below the incision
- A scar in the neck (fades over time)

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Common conditions (most are temporary or resolve with treatment)

- **Temporary weakness of the lower lip** — Due to stretching of the nerve branch that pulls the lower lip down (marginal mandibular nerve) during surgery, about **10–20 in 100** patients have slight asymmetry of the lower lip on that side when smiling or talking. It is a cosmetic issue, usually does not affect eating or drinking, and mostly **improves within weeks to a few months**.
- **Firmness and numbness at the incision** — Firmness and reduced sensation below the incision are common in the first few weeks and lessen over months.

Uncommon conditions

- **Bleeding and haematoma** — In about **1–3 in 100** patients, blood may collect under the skin. If large, it may need to be drained and the bleeding stopped in the operating theatre.
- **Wound infection** — Can occur in **1–3 in 100** patients, especially if the gland was infected; it usually settles with antibiotics, and rarely drainage is needed.
- **Reduced sensation on one side of the tongue** — Due to involvement of the nerve that gives sensation to the tongue (lingual nerve), numbness or tingling on that side of the tongue occurs in about **1–3 in 100** patients. It is mostly temporary; rarely it may be permanent and may also affect taste.
- **Fluid collection (seroma)** — Fluid collecting under the incision; usually settles with needle drainage and a pressure dressing.
- **Stone remaining in the duct** — A stone may remain in, or re-form in, the part of the salivary duct left in the mouth and cause swelling or infection of the floor of the mouth. If necessary, it is removed through the mouth with a small procedure.
- **Noticeable or thick scar** — Depending on individual tendency, the scar may thicken or remain noticeable; scar treatments can be used.
- **Hollowing under the jaw** — A slight hollow where the gland was and a difference between the two sides may develop; it is usually not noticeable.

Rare or very rare, but potentially serious conditions

- **Permanent weakness of the lower lip** — In **about 1–3 in 100** patients or fewer, permanent reduced movement of the lower lip on that side and an asymmetric smile may remain. If noticeable, treatments such as botulinum toxin to balance the other side can be used.
- **Weakness of tongue movement** — Injury to the nerve that moves the tongue (hypoglossal nerve) is **very rare**. If it occurs, there may be weakness of that side of the tongue and difficulty with speech and swallowing; in case of permanent damage, speech and swallowing therapy may be needed.
- **Large neck haematoma affecting the airway** — Very rarely, a rapidly enlarging collection of blood can press on the neck and cause difficulty swallowing or **breathing**. This is an emergency: it must be drained in the operating theatre immediately, and rarely the airway may need to be secured (intubation or tracheostomy).
- **Final pathology different from the preliminary diagnosis** — Even in a gland thought to have stones or infection, the final pathology may show an unexpected tumour. If a malignant tumour is found, further treatment such as removal of the neck lymph nodes or radiotherapy may be needed.
- **Dry mouth** — Removal of one gland goes unnoticed in most patients; however, a dry mouth may become noticeable in patients who have had radiotherapy, have certain rheumatic diseases or take medicines that cause a dry mouth.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.

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- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

9 When to contact us immediately

- **Rapidly increasing swelling of the neck**, especially with **difficulty breathing or swallowing** (emergency — call 112 immediately)
- **Bleeding that does not stop** from the drain or wound
- **Fever above 38.5 °C**, increasing redness, pain, warmth or pus from the wound
- Marked swelling of the tongue or floor of the mouth
- **New, marked difficulty** with speaking or swallowing

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- Keep the wound dry for the first 48 hours; after that you may take a short shower as instructed.
- In the first few days choose soft, lukewarm foods; pay attention to oral hygiene and drink plenty of fluids.
- Protect the scar from the sun for at least 6 months (high-factor sunscreen).
- Make sure to discuss the pathology result and the follow-up plan with your doctor.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

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11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

- ☐ I agree that my images are used **only for medical records and treatment follow-up**.
- ☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).
- ☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.
- ☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write “none”.

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.

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- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that the definitive diagnosis will be made by pathology after surgery and that further treatment may be needed depending on the result.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

- ☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.
- ☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

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This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.